HealthEdu

# Project Inclusion



## Concept Note

### 2025-2026

### Embassy of the People's Republic of China in Rwanda

Presented By: HealthEdu

**Norrsken Kigali, HealthTech Hub, KN78 Street,**
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# Project overview

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|---|---|
| **Name of Organization:** | **HealthEdu** |
| **Project Title** | **Project Inclusion** |
| **Target Country** | Rwanda |
| **Site(s)/Location(s)** | 14 Districts ((Nyabihu, Gicumbi, Karongi, Nyamasheke, Nyamagabe, Nyaruguru, Huye, Gisagara, Nyanza, Ruhango, Kicukiro, Bugesera, Rwamagana, Kayonza) |
| **Number and Type of Beneficiaries:** | • Direct beneficiaries: 244,000<br>• Community Health Workers: 930<br>• Indirect beneficiaries: 500,0000+ from resharing<br>• Community Health Workers<br>• People with rehabilitation needs |
| **Period of Performance:** | 12 Months |
| **Primary Point of Contact:** | Jean Damascene Bigirimana Ngamba |
| **Organization Address:** | Kigali-Rwanda - Norrsken House |
| **Phone Number** | +250 788 898 262 |
| **Funding amounts** | $207,320 |
| **Project Goal:** | To increase equitable access to quality, community-based rehabilitation services in Rwanda by empowering Community Health Workers with digital health tools and knowledge, leveraging the Inclusion App to reduce preventable disabilities, improve functional health outcomes, and strengthen the continuum of care in underserved and hard-to-reach areas |
| **Project Objectives** | To empower 930 community health workers and primary healthcare providers with digital rehabilitation knowledge and digital literacy.<br><br>Facilitate the delivery of 12,000+ tailored exercise programs per month to community members in need of rehabilitation services. |
| **Project Deliverables** | • 930 Community Health workers empowered with digital rehabilitation services and increased digital literacy<br>• 930 smartphones with installed Inclusion App distributed to community health workers<br>• More than 244,000 exercise programs shared to people in rural areas who otherwise couldn't have access to rehabilitation services.<br>• Reduced disability rates and improved quality of life across project sites.<br>• Strengthened health systems at community and primary care levels |



# Executive Summary

Project Inclusion is a digital health and community empowerment initiative designed to improve access to physiotherapy, wellness, and inclusive care for underserved populations in Rwanda. With a focus on musculoskeletal (MSK) conditions With a focus on musculoskeletal (MSK) conditions and disability-friendly health services, the program empowers community health workers (CHWs), caregivers, and individuals with disabilities through technology-enhanced tools and locally adaptable educational content.

It was launched in 2021 by Physiotools Oy (a subsidiary of Physitrack PLC), in partnership with JAMK University of Applied Sciences and local universities in Indonesia and Rwanda and the Inclusion App was developed.

It empowers Community Health Workers (CHWs) to deliver basic, safe and effective rehabilitation services directly within communities. With over 15,000 people already supported and 90% reporting recovery without needing further referral, the Inclusion App has proven to be 18 times more cost-effective than traditional facility-based rehabilitation. In 2025, the Project Inclusion transitioned to a new independent entity: Lumira Link, dedicated to scaling the initiative across ecosystems and countries.

This concept seeks funding to scale the Inclusion App across 14 additional districts, training 930 CHWs through a two-day orientation, distributing pre-installed smartphones, and providing ongoing monthly field supervision and refresher training. The intervention will enable the delivery of over 244,000 personalized exercise programs annually, transforming access to care for thousands of people living in remote areas.

We are calling on partners, donors, and stakeholders to join us in closing Rwanda's rehabilitation gap through digital-first, but not digital only, community-anchored innovation. A total investment of USD 207,320 will empower CHWs, strengthen community health systems, and ensure sustainable, equitable rehabilitation access across Rwanda.

# Background

Over 2.4 billion people worldwide currently need rehabilitation services. In Rwanda, the need is growing rapidly, driven by the increasing burden of non-communicable diseases (NCDs), aging and chronic conditions. Despite this, fewer than 500 Physiotherapists are tasked with serving a population of over 14 million. This imbalance is even more pronounced at the community level, where over 90% of Rwandans access care through primary healthcare facilities, yet rehabilitation professionals are only available at secondary and tertiary levels, and in limited numbers.

The forecasted global rise in rehabilitation needs will not leave Rwanda untouched. As people live longer, but with more chronic conditions and disabilities, the gap between demand and service availability widens. This challenge threatens the Universal Health Coverage (UHC) goal, especially for underserved populations. Without accessible and affordable rehabilitation, rural communities are left behind.

Rwanda's mountainous terrain also intensifies the issue. Daily life often includes walking long distances, climbing steep hills, and engaging in strenuous manual labour, contributing to chronic musculoskeletal pain, injuries, and functional limitations. Without timely early-stage care, these conditions evolve into avoidable disabilities that heavily weigh on individuals and families.

# Our Approach: Inclusion App

Inclusion App is a game-changing digital health solution developed to bridge Rwanda's rehabilitation gap. Co-created with Physiotools and supported by Business Finland, the Inclusion App enables trained Community Health Workers (CHWs) to deliver basic rehabilitation services at the community level, right where people need them most.
This smartphone-based tool offers:

- Professionally designed, contextualized exercise programs for common musculoskeletal conditions (back pain knee pain, etc).
- Short, guided videos narrated in local language.
- Easy content sharing via WhatsApp or in-person demonstrations.
- exercise programs pre-designed by licensed physiotherapists.

Through this approach of "*digital first, not digital only model*", the Inclusion App brings timely, safe and effective rehabilitation interventions to the doorstep of rural communities, therefore dramatically expanding access.

## General Objectives

To increase equitable access to quality, community-based rehabilitation services in Rwanda by empowering Community Health Workers with digital health tools and knowledge, leveraging the Inclusion App to reduce preventable disabilities, improve functional health outcomes, and strengthen the continuum of care in underserved and hard-to-reach areas.



## Specific Objectives

- To empower 930 community health workers and primary healthcare providers with digital rehabilitation knowledge and digital literacy.
- Facilitate the delivery of 12,000+ tailored exercise programs per month to community members in need of rehabilitation services.

## Expected Outcomes

- 930 Community Health workers empowered with digital rehabilitation services and basic digital literacy
- 930 smartphones with installed Inclusion App distributed to community health workers
- More than 800,000 exercise programs shared to people in rural areas who otherwise couldn't have access to rehabilitation services.
- Reduced disability rates and improved quality of life across project sites.
- Strengthened health systems at community and primary care levels

## Solution and Methodology

Since 2023, over 11,000 individuals in Rwanda, who would otherwise have no access to rehabilitation, have received support through the Inclusion App with only 70 CHWs from five health centres selected during the initial pilot phase to test feasibility while delivering future solution today. CHWs have been trained to share tailored, safe exercises for musculoskeletal conditions, with 90% of users reporting meaningful recovery without needing further referral. Moreover, Digital health in service delivery especially using Inclusion App have proven to be 7 times more cost-effective than traditional rehabilitation services and also the Inclusion App pathway achieved significantly higher quality of life impact, with 1.38 QALYs gained per user, compared to the physiotherapy pathway with 0.47 QALYs
gained per patient as reported in Policy Brief for digital rehabilitation services by Kempers J. el 2025.

# Inclusion App Impact



## TESTIMONIAL

I faced debilitating back pain that prevented me from even picking up a spoon from the floor. My situation improved significantly after I attended a community gathering, where a community health worker discussed rehabilitation exercises. She later visited my home and provided me with a basic exercise regimen with her phone. With her consistent support over three months, I regained the ability to perform daily activities. Unlike previous medications that were ineffective, these exercises led to real improvement. Now fully recovered. I actively encourage others in my community facing rehabilitation challenges to seek help from community health workers.



GRATIEN BARASEBYA,
67 YEARS OLD

After meeting community health workers, I was struggling with severe low back pain that made farming impossible without medication. They introduced me to a daily exercise routine, and within a month, I noticed significant improvements and stopped taking medication. With their support, I regained my ability to farm pain-free, much to my wife's astonishment. I continue the exercises and have remained pain-free ever since.

**Inclusion App Impact**



I faced significant back pain that made even the simplest daily activities nearly impossible. I had to rely on a stool in the bathroom just to take a shower while standing. Tasks like doing laundry, sweeping, and mopping were out of reach, and my job as a vegetable seller was affected since I couldn't lift items for my customers.When I met a community health worker, I opened up about my difficulties. She reassured me that there were exercises I could do to help, even though I felt overwhelmed at the time. Gradually, I started these safe and simple exercises, and I began to see improvements. I found it easier to go from lying down to standing."

Ingabire Godelive
42 YEARS OLD

**Inclusion App Impact**



# Methodology

## *Activity 1: Deploying Inclusion App and training of Community Health workers to use Inclusion App*

Following ethical clearance from the National Ethics Committee, we are set to scale the Inclusion App across 14 additional districts (Nyabihu, Gicumbi, Karongi, Nyamasheke, Nyamagabe, Nyaruguru, Huye, Gisagara, Nyanza, Ruhango, Kicukiro, Bugesera, Rwamagana, Kayonza), adding to our existing implementation in Musanze, Burera, and Nyarugenge districts. We will select one health centre with maximum population in their catchment area and train 66 community health workers on use of digital technologies in providing rehabilitation services. Under two days orientation training, a total of 930 community health workers will be trained in seven clusters, with one cluster made of 130 CHWs and equipped with smartphones with already installed Inclusion App.

Inclusion App provides in app short video instructions for each contextualized exercise that was pre-designed by rehabilitation experts for the most prevalent cases in Rwanda such as low back pain, knee pain, elbow pain, hip pain, shoulder pain, neck pain, geriatric exercises, and general well-being exercises.

Training 930 community health workers will not only be vital in ensuring that people living in hard to reach communities where access to rehabilitation services is still a challenge, have accessible, affordable and holistic rehabilitation services, they will also stand as digital rehabilitation voice and agent of change during integrating digital rehabilitation services in normal community health workers workflow during scale up.

## Execution Plan:

- Identify one health centre in every district with high number of population in their catchment area
- Introductory meeting with Health centre management and line referral rehabilitation professional to ensure that digital first model complement, not replace, existing line of help to the community.
- With the head of Health centre manager and Community Health and Environment Officer (CHEO), select at least 66 community health workers from each selected health centre.
- Procure and distribute 930 smartphones with installed Inclusion App to all selected community health workers
- Conduct two-day orientation sessions per site on:
  - Digital rehabilitation service provision.
  - Safe prescription of contextualized exercise programs for conditions like low back pain, knee pain, neck pain, shoulder pain, geriatric cases, and general wellbeing.

## Expected Outcomes



- Two days orientation training conducted per area
- 930 Community health workers trained and empowered with digital rehabilitation service provision knowledge
- 930 smartphones with Inclusion App installed procured and distributed
- Community-level rehabilitation capacity significantly increased.
- Strengthened community to health centres referral system for people with need of rehabilitation beyond digital rehabilitation services offered by community health workers

***Activity 2: Quarterly field visits for each health centre for continued empowerment and monitoring.***

With the digital first model, we ensure that community health workers are constantly empowered and monitored to ensure that they keep delivering quality services to the community, with taking precaution to different red flags, and timely resolving any bug that might arise with the functionality of the Inclusion App.

## Execution Plan

- Conduct one field visit per month for every health centre, meet with all community health workers, conduct one day refresher training on rehabilitative conditions they are sharing exercises for with Inclusion.
- Troubleshoot and resolve any app-related issues.
- Ensure ongoing mentorship and data quality checks.

## Expected Outcomes

- Consistent quality of digital rehabilitation services
- Improved CHW confidence and service delivery.
- Resolved technical issues, ensuring app functionality remains user-friendly.

# Monitoring and Evaluation Plan

Our M&E strategy includes a mix of quantitative and qualitative tools: In-app analytics on exercises shared and user engagement, field visit reports and refresher training logs, CHW feedback and supervision forms, surveys assessing reduction in rehab-related facility visits, success stories and testimonials from beneficiaries, etc.

By closely tracking our indicators, we ensure alignment with project goals while generating rich evidence for scale-up and policy advocacy. Notably, Inclusion App's cost-effectiveness, 7 times less expensive than traditional services, positions it as a sustainable model for digital health in low-resource settings (Kempers J. et al., 2025).

# Sustainability Plan

The sustainability of Inclusion lies in its uniqueness and novelty, knowledge transfer through training community health workers, and most importantly striving to mainstreaming Inclusion into the normal workflow of the community health workers as the Inclusion App aligns well with national strategic plan V and sustainable Development Goals 3.

# Call for Action

We are actively seeking partners like the Embassy of People's Republic of China who share our vision of equitable, innovative, and scalable health solutions. Your support of USD $207,320 will allow us to equip 930 Community Health Workers, reach the unreached with 12,000 exercise programs monthly, and scale this impactful intervention across Rwanda.

# Gross Budget

| HealthEdu | | |
|---|---|---|
| **Project Inclusion** | | |
| **Budget Request: 290,249,400 ($207,320 USD)** | | |
| **Main Activity** | **Total Amount in RWF** | **Total Amount In USD** |
| Act 1: Deploying Inclusion App and training of Community Health workers to use Inclusion App | 118,584,000 | 84,702 |
| Act 2: Quarterly field visits for each health centre for continued empowerment and monitoring | 67,200,000 | 48,000 |
| Monitoring and Evaluation | 17780000 | 12,700 |
| Project Overheads | 34,800,000 | 24,857 |
| Project Logistics and administration fees | 38,064,000 | 27,188 |
| **Grand-Total** | **276,428,000** | **197,448** |
| **Contingency 5% of grand-total** | **13821400** | **9,872** |
| **Total Budget** | ***290,249,400*** | ***207,320*** |

HealthEdu™

# Thank You

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# Value Proposition

. By equipping Community Health Workers with digital tools to deliver essential rehabilitation services at the grassroots level, the project not only addresses critical health inequities and contributes to SDG 3 (Good Health and Well-being) and SDG 10 (Reduced Inequalities), but also significantly reduces financial strain on vulnerable households, directly contributing to SDG 1 (No Poverty).

By restoring mobility and functional independence, it enables individuals to return to productive activities and livelihoods, fostering decent work and economic growth at the community level (SDG 8).

This digital-first, but not digital-only, model strengthens Rwanda's primary healthcare system by building a sustainable, functional, and effective community health workforce, demonstrating the power of digital connectivity for tangible, long-term impact. Supporting the scale-up of the Lumiralink App will amplify the impact on Rwandan communities, foster health equity, and showcase the strong bilateral cooperation between China and Rwanda in advancing inclusive digital health innovation.